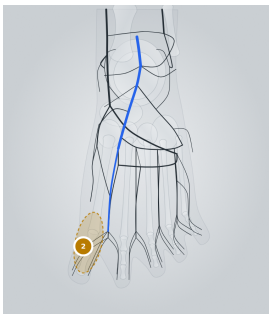
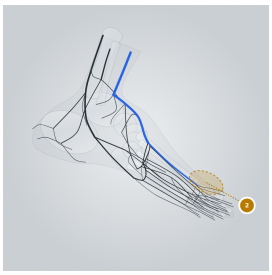
CLINICAL INDICATION



AP (dorsoplantar)



Lateral oblique

Colors mark selected wound locations; blue marks the planned pedal route.

Fontaine stage
IV

WIfI
W2 I2 fI1 - clinical stage 4

Estimated 1-year major amputation risk
approximately 38% (pooled observational estimate)

Offer revascularisation after assessment of patient risk, anatomy and conduit.

PAT: Dorsalis pedis artery - 260 ms - Category 4 - severe ischemia

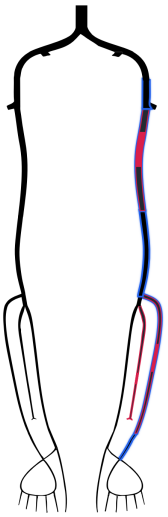
PAT: Lateral plantar artery - 200 ms - Category 3 - moderate ischemia

Wound location
Hallux / first ray

Wound-informed target arterial path to consider
Anterior tibial -> dorsalis pedis target arterial path or Posterior tibial -> plantar arch target arterial path

Confirm direct wound-bed filling, pedal arch and collateral quality with AP and lateral foot angiography.

DISEASE ANATOMY



Disease anatomy

Red: lesion outline / estimated extent. Blue: target path. Schematic position.

Lesion 1 - Left superficial femoral artery
occlusion, 12 cm, heavy calcification, schematic middle position

Lesion 2 - Left anterior tibial artery
occlusion, 6 cm, moderate calcification, schematic middle position

Lesion 3 - Left peroneal / fibular artery
stenosis, 3 cm, moderate calcification, schematic middle position

Target path
common femoral artery -> superficial femoral artery -> popliteal artery -> anterior tibial artery -> dorsalis pedis artery -> first dorsal metatarsal artery

GLASS III
FP 4, IP 3, P0

Technical failure >20%; 1-year limb-based patency <50%

Extensive femoropopliteal or infrapopliteal occlusions, disease in both segments, or popliteal chronic total occlusion.

INTERVENTION PLAN

ACCESS

Approach 1
Antegrade Left CFA

Needle
21 Gauge | 7cm

Sheath
BD Interventional - Halo One Thin-Walled Guiding Sheath · 0.018-inch · 6 Fr · 25 cm

Catheter
Navicross 0.018 · 2.6 Fr · 135 cm

NAVIGATION & CROSSING

Lesion 1 Left superficial femoral artery - wire
Asahi Intecc - Gladius MG PV · 0.018-inch · 300 cm · CTO crossing · Jacketed / sliding · Intraluminal tracking · Straight

Lesion 3 Left peroneal / fibular artery - wire
Abbott - Hi-Torque Command 18 LT · 0.018-inch · 300 cm · Workhorse · Intraluminal tracking · Straight

Lesion 2 Left anterior tibial artery - wire
Asahi Intecc - Gladius MG PV · 0.018-inch · 300 cm · CTO crossing · Jacketed / sliding · Intraluminal tracking · Straight

VESSEL PREPARATION & THERAPY

Lesion 1 Left superficial femoral artery - balloon
Cook Medical - Advance 18LP Low Profile PTA Balloon Catheter · Standard PTA · 0.018-inch · 5 x 150 mm · shaft 135 cm · Over-the-wire · minimum sheath 5 Fr

Lesion 3 Left peroneal / fibular artery - balloon
Cook Medical - Advance 18LP Low Profile PTA Balloon Catheter · Standard PTA · 0.018-inch · 3 x 40 mm · shaft 135 cm · Over-the-wire · minimum sheath 4 Fr

Lesion 2 Left anterior tibial artery - balloon
Cook Medical - Advance 18LP Low Profile PTA Balloon Catheter · Standard PTA · 0.018-inch · 3 x 80 mm · shaft 135 cm · Over-the-wire · minimum sheath 4 Fr

CLOSURE

Closure
Manual pressure

RECOMMENDATIONS & CONSIDERATIONS

Recommendation | Document calcium modification and bailout strategy
Left superficial femoral artery is heavily calcified, but no dedicated calcium-modification strategy is recorded. Consider whether prolonged appropriately sized angioplasty, scoring/cutting technology, intravascular lithotripsy, or atherectomy is appropriate, and define the dissection/recoil bailout plan.

MATERIALS PREPARATION

Tick when prepared. Enter quantity in QTY. Repeated plan entries may use the same device.
Confirm quantities and reuse before opening; only selected case materials are listed.

01 ACCESS & INTRODUCERS

☐ **Access needle**
21 Gauge | 7cm
Use: Access 1: Antegrade Left CFA
QTY

☐ **BD Interventional - Halo One Thin-Walled Guiding Sheath**
6 Fr | 25 cm | 0.018-inch
Use: Access 1: Antegrade Left CFA
QTY

02 GUIDEWIRES

☐ **Asahi Intecc - Gladius MG PV**
300 cm | 0.018-inch
Tip Straight | CTO crossing | Jacketed / sliding
Use: Crossing / support - Lesion 1; Crossing / support - Lesion 2
2 selected plan entries - confirm whether shared or separate units.
QTY

☐ **Abbott - Hi-Torque Command 18 LT**
300 cm | 0.018-inch
Tip Straight | Workhorse
Use: Crossing / support - Lesion 3
QTY

03 CATHETERS & SUPPORT

☐ **Navicross 0.018**
2.6 Fr | 135 cm
Use: Access 1: Antegrade Left CFA
QTY

04 BALLOONS

☐ **Cook Medical - Advance 18LP Low Profile PTA Balloon Catheter**
5 x 150 mm | 0.018-inch
Shaft 135 cm | Minimum sheath 5 Fr | Over-the-wire | Standard PTA
Use: Treatment - Lesion 1
QTY

☐ **Cook Medical - Advance 18LP Low Profile PTA Balloon Catheter**
3 x 40 mm | 0.018-inch
Shaft 135 cm | Minimum sheath 4 Fr | Over-the-wire | Standard PTA
Use: Treatment - Lesion 3
QTY

MATERIALS PREPARATION

Tick when prepared. Enter quantity in QTY. Repeated plan entries may use the same device.
Confirm quantities and reuse before opening; only selected case materials are listed.

04 BALLOONS (continued)

☐ **Cook Medical - Advance 18LP Low Profile PTA Balloon Catheter**
3 x 80 mm | 0.018-inch
Shaft 135 cm | Minimum sheath 4 Fr | Over-the-wire | Standard PTA
Use: Treatment - Lesion 2

QTY

05 CLOSURE & HAEMOSTASIS

☐ **Manual pressure**
Use: Closure 1